

Australian MMI Question Bank — FREE SAMPLE

Original practice material for Australian medical and dental school MMIs. Not affiliated with any university, UCAT ANZ, GAMSAT, or admissions body. For general practice only.

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What is an MMI?

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The Multiple Mini Interview (MMI) is a timed circuit of 6–10 short 'stations'. At each you enter a room with a prompt and/or actor (patient), have ~2 minutes to read and think, then ~5–8 minutes to respond. Assessors score you at each station against the domain it tests (communication, ethics, teamwork, etc.). It's used by most Australian medical and dental schools.

Why it exists: standardised, repeated-measures assessment of the non-cognitive attributes that matter in practice — communication, empathy, judgement, resilience — that interviews and ATAR/UCAT scores don't capture.

The golden rule: the assessors are not looking for a 'right answer'. They're looking for HOW you think, how you treat the person in front of you, and how you handle pressure. Structure + empathy beats a 'perfect' answer.

A universal structure for any station: PAUSE (breathe, read twice, identify the core demand) → POINTS (the 2–3 key issues) → ACTION (what you'd actually say/do, in order) → REFLECT (what you'd watch for / revisit).

8-week preparation plan

- Weeks 1–2 · Foundations: Read every station's scenario + thinking framework. Don't answer aloud yet — build the mental toolkit. Do 2 stations/day, 15 min each.
- Weeks 3–4 · Structured answers: Answer each station aloud using ONLY the framework (pause, structure, answer). Record yourself. Aim for 60–90 seconds of structured speech per station.
- Weeks 5–6 · Role-play: Partner up. Run the communication/role-play stations live with a timer (5–7 min). Partner plays the examiner and asks the follow-ups.
- Weeks 7–8 · Full mock MMI: Assemble 6–8 stations into a timed circuit. 2 min prep + 5 min answer per station, 1 min rest between. Do this at least twice.
- ALL-WEEK: Keep a reflection journal after each station — what felt weak, what phrase worked, what you'd change.

How stations are scored

- 1–2 (Weak): No structure; emotion-driven; misses the station's core demand; poor listening in role-play.
- 3 (Basic): Some structure; one or two good points; struggles with follow-ups; answers not tailored to the patient.
- 4 (Good): Clear structure (pause–points–action); addresses the person first; realistic and specific; handles one follow-up well.
- 5 (Excellent): Empathy + ethics + action all land; natural, specific, patient-centred; calmly handles all follow-ups; shows genuine self-awareness.

Ethics & Professionalism

You are a student on clinical placement. You overhear a nurse criticising a patient loudly in the corridor, using unprofessional language about the patient's weight.

Task: You are about to enter the lift with this nurse. What do you do, and what principles guide your response?

✓ THINKING FRAMEWORK

1. Assess immediate risk — is the patient harmed or distressed right now? 2. Address the nurse respectfully and privately, not in public confrontation. 3. Distinguish between immediate duty (patient dignity) and reporting duty (professional standards). 4. Use the principles: respect for dignity, non-maleficence, professional accountability. 5. Escalate through appropriate channels if the behaviour continues (clinical educator, supervisor).

△ COMMON PITFALLS

- Confronting the nurse loudly in front of others (escalates conflict, embarrasses both)
- Doing nothing ('it's not my place') — passive complicity
- Jumping straight to formal reporting without addressing it once
- Focusing only on the nurse and forgetting the patient's perspective

→ FOLLOW-UP QUESTIONS

- The nurse defends their behaviour, saying the patient 'doesn't care'. How do you respond?
- What if the nurse is your direct supervisor?
- When is it appropriate to formally report vs. informally address?

★ EXAMINER TIP

- Balance courage with tact; show you understand the hierarchy but don't let it silence you.

Communication & Role-Play

ROLE-PLAY: You are a dental student and a patient (played by the examiner) has just been told they need a tooth extraction. The patient is anxious and says: 'I really don't want this. Isn't there another way? I'm scared of the pain.'

Task: Respond to the patient in the role-play. Address their concern and build trust.

✓ THINKING FRAMEWORK

1. Acknowledge the emotion first ('I can see you're really anxious about this — that's completely understandable'). 2. Explore the source of fear (pain, needles, loss of tooth, past bad experience). 3. Explain options honestly (root canal, alternatives, why extraction may be indicated). 4. Explain pain management (local anaesthetic, gentle technique, post-op care). 5. Involve them in the decision — it's their mouth.

△ COMMON PITFALLS

- Jumping straight to clinical justification without acknowledging fear
- Making promises you can't keep ('it won't hurt at all')
- Minimising the patient's feelings ('it's not that bad')
- Using jargon the patient doesn't understand

→ FOLLOW-UP QUESTIONS

- The patient says a relative had a horrible extraction. How does this change your approach?
- The patient agrees but asks what happens 'if it goes wrong'. Respond.
- How do you communicate risk honestly without frightening the patient further?

★ EXAMINER TIP

- Prioritise empathy before explanation. In role-plays, examiners reward listening, warmth, and shared decision-making — not clinical dumping.

Why do you want to study [dentistry/medicine]? What is your genuine motivation?

Task: Answer honestly, with a story or example.

✓ **THINKING FRAMEWORK**

1. Be specific to YOU — a story, experience, or person that shaped you. 2. Connect it to what the profession actually involves (not just 'helping people' generically). 3. Show you've considered the reality: the study load, the patient relationships, the daily realities. 4. Acknowledge a challenge you've seen or anticipate. 5. End with what you'll contribute.

△ **COMMON PITFALLS**

- Generic answers ('I love helping people' / 'job security')
- Sounding rehearsed or reciting a script
- Not mentioning any understanding of the profession's realities
- Avoiding any self-reflection

→ **FOLLOW-UP QUESTIONS**

- What do you think is the hardest part of being a dentist/doctor?
- What will you do if you don't get in this year?
- Why this school/university specifically?

★ **EXAMINER TIP**

- Examiners have heard every cliché. A specific, humble, self-aware answer stands out.

A patient with significant dental anxiety and a strong gag reflex needs a filling. They've postponed treatment for years and are only here now because of pain.

Task: How do you manage this patient?

✓ **THINKING FRAMEWORK**

1. Acknowledge and validate the anxiety — this took courage. 2. Build trust slowly; don't rush to the chair. 3. Discuss options: local anaesthetic techniques, desensitisation, sedation options (nitrous oxide/IV sedation where indicated), breaks during treatment. 4. Agree a 'stop signal' — give the patient control. 5. Consider a staged approach (build up over visits).

△ **COMMON PITFALLS**

- Dismissing the gag reflex as 'manageable' without a plan
- Scheduling a full filling without preparing the patient
- Ignoring the years of avoidance and the fear behind it
- Not offering control mechanisms (stop signal, breaks)

→ **FOLLOW-UP QUESTIONS**

- The patient is fine at the first visit but anxious at the next. How do you maintain momentum?
- How do you balance patient comfort with treatment efficiency?
- What does a 'patient-centred' approach to anxiety look like?

★ **EXAMINER TIP**

- Dental-specific stations test whether you understand the whole person — fear, trust, and control — not just the tooth.

Dental-Specific Scenario

A 12-year-old patient comes in with their parent. The parent wants a 'perfect smile' and requests bleaching for the child. The child is reluctant and embarrassed.

Task: How do you respond?

✓ THINKING FRAMEWORK

1. Consider the child's autonomy and wishes — they're the patient. 2. Note clinical appropriateness (bleaching in young permanent teeth, pulp size, evidence base). 3. Address the parent's motivation — is it about the child or the parent's expectations? 4. Have the conversation with the child's voice centred. 5. Offer evidence-based, age-appropriate options and explain the reasoning.

△ COMMON PITFALLS

- Doing what the parent wants without considering the child
- Lecturing the parent
- Ignoring the clinical/evidence considerations
- Not recognising the potential body-image/self-esteem issues

→ FOLLOW-UP QUESTIONS

- What's the ethical tension between parental authority and child autonomy?
- How would you handle a parent pressuring for unnecessary treatment?
- What are the clinical risks of bleaching young teeth?

★ EXAMINER TIP

- This tests paediatric ethics — the child is the patient, not the parent, but both must be respected.

Feedback & Reflection

Your supervisor gives you critical feedback on your clinical performance that you feel is unfair.

Task: How do you respond and what do you do?

✓ THINKING FRAMEWORK

1. Listen fully first — don't defend in the moment. 2. Ask clarifying questions to understand the specific concern. 3. Separate the emotion (feeling unfair) from the content (is there a valid point?). 4. Reflect before responding if needed ('let me think about that and come back to you'). 5. Act on what's valid, and if truly incorrect, raise it professionally later.

△ COMMON PITFALLS

- Getting defensive immediately
- Accepting everything without reflection (no self-advocacy)
- Dwelling on the 'unfairness' and ignoring the learning
- Failing to follow up or demonstrate improvement

→ FOLLOW-UP QUESTIONS

- What's the difference between defensiveness and self-advocacy?
- How do you ask for feedback proactively?
- Describe a time feedback changed your practice.

★ EXAMINER TIP

- Reflective practitioners grow from criticism — show you can metabolise it, not just endure it.